



Susvimo™ PI Explorer

CONFIDENTIAL. FOR INTERNAL TRAINING PURPOSES ONLY.

Susvimo™ PI Explorer

Table of Contents

Note to Reviewers 2

How to Use PI Explorer 3

PI Page 1 Annotations 5

PI Page 2 7

PI Page 2 Annotations 8

PI Page 2 Glossary Terms 10

PI Page 2 Flashcard Questions 11

PI Page 2 Flashcard Answers 12

Note to Reviewers

The SUSVIMO PI Explorer is an interactive program that will allow field representatives to study the full SUSVIMO prescribing information (PI). The text of the PI is accompanied by clickable icons that highlight and reveal additional information relevant to SUSVIMO, including Clinical Concepts, Key Points, Dosing Notes, Efficacy Notes, Safety Notes, and Procedural Notes. Note that the text of the PI itself remains unchanged. Self-check learning exercises are also interspersed throughout the program.

The mock-up screen below provides a visual representation of a screen of the finished PI Explorer. Field representatives are able to page through the SUSVIMO PI in its entirety. Each page includes instructional call-outs indicated by icons (eg, Clinical Concept, Key Point), along with an icon key at the top of the page.

Example: Clicking the Safety Note icon shown on the left brings up the call-out box shown on the right.

HIGHLIGHTS OF PRESCRIBING INFORMATION

These highlights do not include all the information about this medication that may be important to you or your healthcare provider. For full prescribing information, see the full prescribing information for this medication.

SUSVIMO (ranibizumab injection) is a prescription medicine used to treat certain types of eye disease.

Initial Use: The recommended dose of SUSVIMO (ranibizumab injection) is 2 mg (0.02 mL of 100 mg/mL solution) continuously delivered via the SUSVIMO implant with refills every 24 weeks (approximately 6 months).

Supplemental treatment: Supplemental treatment with 0.5 mg intravitreal ranibizumab injection may be administered in the affected eye if clinically necessary.

Implantation: Perform the initial implantation, refill-exchange, and implant removal (if necessary) procedures under strict aseptic conditions.

DOSAGE FORMS AND STRENGTHS

SUSVIMO (ranibizumab injection) is available as a 100 mg/mL solution in a 0.02 mL vial.

Key Point

DOSAGE AND ADMINISTRATION

- For intravitreal use via SUSVIMO ocular implant. (2.1)
- The recommended dose of SUSVIMO (ranibizumab injection) is 2 mg (0.02 mL of 100 mg/mL solution) continuously delivered via the SUSVIMO implant with refills every 24 weeks (approximately 6 months). (2.2)
- Supplemental treatment with 0.5 mg intravitreal ranibizumab injection may be administered in the affected eye if clinically necessary. (2.3)
- Perform the initial implantation, refill-exchange, and implant removal (if necessary) procedures under strict aseptic conditions. (2.4, 2.5, 2.6, 2.7)

This information is discussed in more detail in **Section 2** of the PI.

- For intravitreal use via SUSVIMO ocular implant. (2.1)
- The recommended dose of SUSVIMO (ranibizumab injection) is 2 mg (0.02 mL of 100 mg/mL solution) continuously delivered via the SUSVIMO implant with refills every 24 weeks (approximately 6 months). (2.2)
- Supplemental treatment with 0.5 mg intravitreal ranibizumab injection may be administered in the affected eye if clinically necessary. (2.3)
- Perform the initial implantation, refill-exchange, and implant removal (if necessary) procedures under strict aseptic conditions. (2.4, 2.5, 2.6, 2.7)

conjunctival hyperemia (26%), iritis (23%) and eye pain (10%) (6.1)

To report SUSPECTED ADVERSE REACTIONS, contact Genentech at 1-888-835-2555 or FDA at 1-800-FDA-1088 or www.fda.gov/medwatch.

See 17 for PATIENT COUNSELING INFORMATION and Medication Guide.

Revised: 10/2021

Please also note that all images in the following storyboard are for placement only and will be finalized after initial review.

Susvimo™ PI Explorer

How to Use PI Explorer

The SUSVIMO PI Explorer is intended to increase your understanding of SUSVIMO by presenting in-depth information within the full SUSVIMO prescribing information (PI). This training tool is for internal use only.

A variety of call-out icons are located throughout the PI Explorer. Clicking on the icons reveals additional information in a pop-up window.

-  **Clinical Concept:** provides background clinical information explaining why a PI section may be important to a healthcare provider (HCP)
-  **Key Point:** describes how to use information in the PI to support selling messages delivered in the field
-  **Dosing Note:** provides more in-depth information on dosing of SUSVIMO that may be important to HCPs
-  **Efficacy Note:** provides more information on clinical studies cited in the PI
-  **Safety Note:** explains key safety information
-  **Procedural Note:** provides more in-depth information on SUSVIMO implant procedures that may be important to HCPs

Glossary terms are highlighted within the PI and their definitions can be reviewed in each section following the PI page.

This guide does not represent the full extent of the information you should know from the PI. It is important that you read through the PI completely.

Susvimo™ PI Explorer

PI Page 1

1	HIGHLIGHTS OF PRESCRIBING INFORMATION These highlights do not include all the information needed to use SUSVIMO safely and effectively. See full prescribing information for SUSVIMO. SUSVIMO™ (ranibizumab injection) for intravitreal use via SUSVIMO ocular implant Initial U.S. Approval: 2006	DOSAGE FORMS AND STRENGTHS Injection: 100 mg/mL solution in a single-dose vial (3) CONTRAINDICATIONS - Ocular or periocular infections (4.1) - Active intraocular inflammation (4.2) - Hypersensitivity (4.3)	5
2	WARNING: ENDOPTHALMITIS <i>See full prescribing information for complete boxed warning.</i> The SUSVIMO implant has been associated with a 3-fold higher rate of endophthalmitis than monthly intravitreal injections of ranibizumab. In clinical trials, 2.0% of patients receiving an implant experienced an episode of endophthalmitis.	WARNINGS AND PRECAUTIONS - The SUSVIMO implant and/or implant-related procedures have been associated with endophthalmitis, rhegmatogenous retinal detachment, implant dislocation, septum dislodgement, vitreous hemorrhage, conjunctival retraction, conjunctival erosion, and conjunctival bleb. Patients should be instructed to report signs or symptoms that could be associated with these events without delay. Additional surgical and/or medical management may be required. (5.1, 5.2, 5.3, 5.4, 5.5, 5.6, 5.7) Vitreous Hemorrhage: Temporarily discontinue antithrombotic medication prior to the implant insertion procedure to reduce the risk of vitreous hemorrhage. Vitrectomy may be needed. (5.5) Postoperative Decrease in Visual Acuity: A decrease in visual acuity usually occurs over the first two postoperative months. (5.8)	6
3	RECENT MAJOR CHANGES Dosage and Administration (2.4, 2.7) 4/2022 Warnings and Precautions (5.4) 4/2022 INDICATIONS AND USAGE SUSVIMO (ranibizumab injection), a vascular endothelial growth factor (VEGF) inhibitor, is indicated for the treatment of patients with Neovascular (wet) Age-related Macular Degeneration (AMD) who have previously responded to at least two intravitreal injections of a VEGF inhibitor (1.0).	ADVERSE REACTIONS The most common adverse reactions were conjunctival hemorrhage (72%), conjunctival hyperemia (26%), iritis (23%) and eye pain (10%) (6.1) To report SUSPECTED ADVERSE REACTIONS, contact Genentech at 1-888-835-2555 or FDA at 1-800-FDA-1088 or www.fda.gov/medwatch. See 17 for PATIENT COUNSELING INFORMATION and Medication Guide. Revised: 4/2022	7
4	DOSAGE AND ADMINISTRATION - For intravitreal use via SUSVIMO ocular implant. (2.1) - The recommended dose of SUSVIMO (ranibizumab injection) is 2 mg (0.02 mL of 100 mg/mL solution) continuously delivered via the SUSVIMO implant with refills every 24 weeks (approximately 6 months). (2.2) - Supplemental treatment with 0.5 mg intravitreal ranibizumab injection may be administered in the affected eye if clinically necessary. (2.3) - Perform the initial implantation, refill-exchange, and implant removal (if necessary) procedures under strict aseptic conditions. (2.4, 2.5, 2.6, 2.7)		8

FULL PRESCRIBING INFORMATION: CONTENTS*

WARNING: ENDOPTHALMITIS

1 INDICATIONS AND USAGE

2 DOSAGE AND ADMINISTRATION

- 2.1 General Information
- 2.2 Neovascular (Wet) Age-Related Macular Degeneration (AMD)
- 2.3 Supplemental Treatment with Intravitreal Ranibizumab Injection
- 2.4 Ocular Implant Initial Fill
- 2.5 Ocular Implant Insertion
- 2.6 Ocular Implant Removal
- 2.7 Ocular Implant Refill-Exchange Procedure
- 2.8 Delayed or Missed doses
- 2.9 Dosage (Refill-Exchange) Modifications for Adverse Reactions

3 DOSAGE FORMS AND STRENGTHS

4 CONTRAINDICATIONS

- 4.1 Ocular or Periocular Infections
- 4.2 Active Intraocular Inflammation
- 4.3 Hypersensitivity

5 WARNINGS AND PRECAUTIONS

- 5.1 Endophthalmitis
- 5.2 Rhegmatogenous Retinal Detachment
- 5.3 Implant Dislocation
- 5.4 Septum Dislodgement
- 5.5 Vitreous Hemorrhage
- 5.6 Conjunctival Erosion or Retraction
- 5.7 Conjunctival Bleb
- 5.8 Postoperative Decrease in Visual Acuity

5.9 Air Bubbles Causing Improper Filling of the Implant

5.10 Deflection of Implant

6 ADVERSE REACTIONS

- 6.1 Clinical Trials Experience
- 6.2 Immunogenicity

8 USE IN SPECIFIC POPULATIONS

- 8.1 Pregnancy
- 8.2 Lactation
- 8.3 Females and Males of Reproductive Potential
- 8.4 Pediatric Use
- 8.5 Geriatric Use

11 DESCRIPTION

12 CLINICAL PHARMACOLOGY

- 12.1 Mechanism of Action
- 12.3 Pharmacokinetics

13 NONCLINICAL TOXICOLOGY

- 13.1 Carcinogenesis, Mutagenesis, Impairment of Fertility

14 CLINICAL STUDIES

16 HOW SUPPLIED/STORAGE AND HANDLING

- 16.1 How Supplied
- 16.2 Storage
- 16.3 Handling

17 PATIENT COUNSELING INFORMATION

* Sections or subsections omitted from the full prescribing information are not listed.

Susvimo™ PI Explorer

PI Page 1 Annotations



Key Point 1

The SUSVIMO PI begins with the “Highlights of Prescribing Information” section. According to the US Food and Drug Administration (FDA), the Highlights section of the PI is designed to give practitioners immediate access to the information they refer to most often and regard as most important. This section also directs the practitioner to the section in the full PI where they can obtain details about a specific topic.

Sections 1 through 3 are related to indications, dosage, and administration, and sections 4 through 8 are related to safety.

Note that while this section points out the most crucial elements of the PI, it does not contain all the information needed to safely and effectively prescribe and use SUSVIMO. Therefore, it is important that providers understand the content of the full PI.



Key Point 2

SUSVIMO carries a boxed warning for endophthalmitis. The complete boxed warning is detailed in the full prescribing information.



Key Point 3

This information is discussed in more detail in **Section 1** of the PI.



Key Point 4

This information is discussed in more detail in **Section 2** of the PI.



Key Point 5

This information is discussed in more detail in **Section 3** of the PI.



Key Point 6

This information is discussed in more detail in **Section 4** of the PI.

Susvimo™ PI Explorer

PI Page 1 Annotations



Key Point 7

This information is discussed in more detail in **Section 5** of the PI.



Key Point 8

This information is discussed in more detail in **Section 6** of the PI.

Susvimo™ PI Explorer

PI Page 2

FULL PRESCRIBING INFORMATION

WARNING: ENDOPHTHALMITIS

The SUSVIMO implant has been associated with a 3-fold higher rate of endophthalmitis than monthly intravitreal injections of ranibizumab. Many of these events were associated with conjunctival retractions or erosions. Appropriate conjunctiva management and early detection with surgical repair of conjunctival retractions or erosions may reduce the risk of endophthalmitis. In clinical trials, 2.0% of patients receiving a ranibizumab implant experienced at least one episode of endophthalmitis [see *Contraindications (4.1), Warnings and Precautions (5.1)*].

1 INDICATIONS AND USAGE

SUSVIMO (ranibizumab injection) is indicated for the treatment of patients with Neovascular (wet) Age-related Macular Degeneration (AMD) who have previously responded to at least two intravitreal injections of a Vascular Endothelial Growth Factor (VEGF) inhibitor medication.

2 DOSAGE AND ADMINISTRATION

2.1 General Information

For Intravitreal Use via SUSVIMO ocular implant.

The SUSVIMO initial fill and ocular implant insertion and implant removal procedures must be performed under aseptic conditions by a physician experienced in vitreoretinal surgery. The SUSVIMO ocular implant must be surgically implanted in the eye or removed from the eye (if medically necessary) in an operating room using aseptic technique. See SUSVIMO Instructions for Use and the standardized steps to optimize surgical outcomes.

SUSVIMO refill-exchange procedures must be performed under aseptic conditions by a physician experienced in ophthalmic surgery [see *Dosage and Administration (2.7)*].

Do not administer SUSVIMO (ranibizumab injection) as a bolus intravitreal injection. Do not substitute SUSVIMO (ranibizumab injection) with other ranibizumab products.

Initial Fill: One SUSVIMO initial fill needle (34-gauge, with integrated 5 µm filter and blue cap) is included. A 5-micron sterile filter needle (19-gauge x 1½ inch), and a 1 mL Luer lock syringe are needed but **not included**.

Refill-Exchange: One SUSVIMO refill needle (34-gauge with integrated 5 µm filter and clear cap) is included. A 5-micron sterile filter needle (19-gauge x 1½ inch), and a 1 mL Luer lock syringe are needed but **not included**.

2.2 Neovascular (Wet) Age-Related Macular Degeneration (AMD)

The recommended dose of SUSVIMO (ranibizumab injection) is 2 mg (0.02 mL of 100 mg/mL solution) continuously delivered via the SUSVIMO ocular implant with refills administered every 24 weeks (approximately 6 months).

2.3 Supplemental Treatment with Intravitreal Ranibizumab Injection

Susvimo™ PI Explorer

PI Page 2 Annotations



Safety Note 1

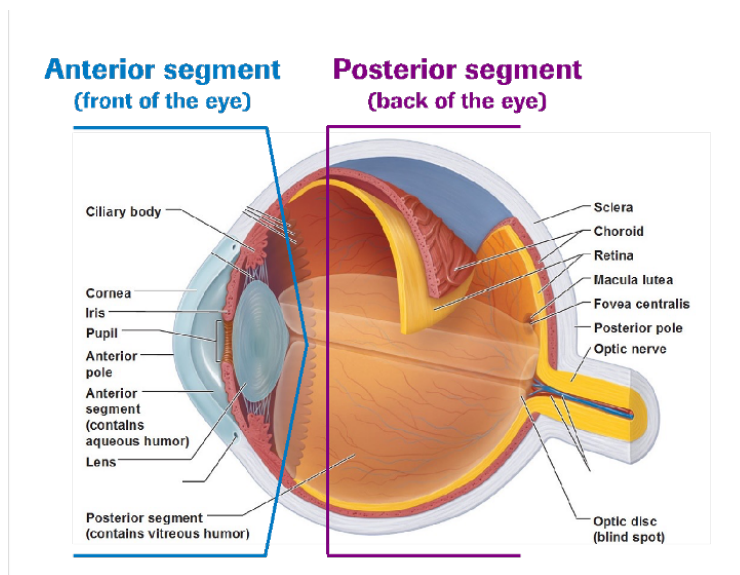
The SUSVIMO implant has a boxed warning for endophthalmitis (inflammation associated with an infection inside of the eye) due to the association with a 3-fold higher risk compared to monthly intravitreal injections of ranibizumab. It is recommended that appropriate conjunctiva management and early detection and repair of conjunctival retraction or erosion may lower the risk of endophthalmitis.



Clinical Concept 2

SUSVIMO is indicated for the treatment of patients with neovascular (wet) age-related macular degeneration (AMD), commonly referred to as nAMD, who have previously responded to ≥ 2 intravitreal injections of a vascular endothelial growth factor (VEGF) inhibitor medication.

AMD is a degenerative disease that begins by affecting the retinal pigment epithelium of the macula (shown in the image below). This condition typically appears in patients aged 60 to 90. It is responsible for 8.5% of cases of blindness globally and affects 1.75 million people in the US. SUSVIMO is indicated for treatment of the neovascular (wet) form of AMD. In nAMD, frail new blood vessels form and may enter the subretinal space (the area beneath the retina). Hemorrhage, or leakage of these blood vessels, causes loss of photoreceptors. This process may result in loss of central vision acuity (loss of the ability to see things directly in front of the eye). In patients with nAMD, both eyes are typically affected in a similar manner.



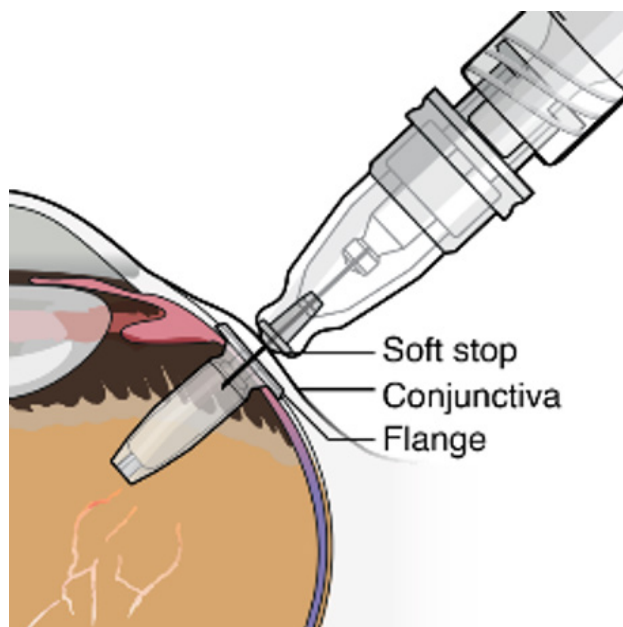
Susvimo™ PI Explorer

PI Page 2 Annotations



Clinical Concept 3

Patients should be selected after ensuring previous response to ≥ 2 intravitreal injections of an anti-VEGF treatment. Intravitreal injection refers to a procedure that uses a needle to place a medication directly into the space in the back of the eye, known as the vitreous cavity, which is filled with a jelly-like fluid referred to as the vitreous humor gel. Typically, this procedure is performed in the office setting by a specially trained ophthalmologist. Anti-VEGF products indicated for intravitreal injection that are currently available in the US include LUCENTIS® (ranibizumab) and Eylea® (aflibercept).



Dosing Note 4

SUSVIMO is for intravitreal use via the SUSVIMO ocular implant only. It should not be substituted for other products, and other intravitreal products should not be used with the implant. SUSVIMO procedures—including implant insertion, implant removal, and refill-exchange—should be performed by an ophthalmologist trained in these procedures. Implant insertion and removal should only be done by an ophthalmologist experienced in vitreoretinal surgery. SUSVIMO should not be administered as a bolus intravitreal injection.

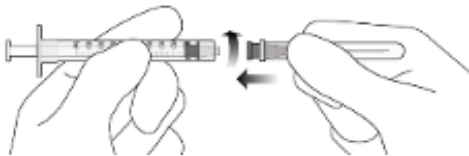


Dosing Note 5

The recommended dose of SUSVIMO is 2 mg (0.02 mL of 100 mg/mL solution), which is continuously delivered via the SUSVIMO ocular implant. Refills are administered every 24 weeks, or approximately every 6 months.

Susvimo™ PI Explorer

PI Page 2 Glossary Terms

GLOSSARY TERMS
aseptic method used in surgery to prevent contamination of the wound and operative site; all instruments used are sterilized, and physicians and nurses wear caps, masks, shoe coverings, sterile gowns, and gloves
conjunctival erosion full thickness degradation or breakdown of the conjunctiva in the area of the implant flange; associated with an increased risk of endophthalmitis
conjunctival retraction recession or opening of the limbal and/or radial peritomy; associated with an increased risk of endophthalmitis
endophthalmitis intraocular infection; may be acute, subacute, or chronic; localized or involving anterior and posterior segments
intravitreal cavity enclosed within the posterior segment of the eye
Luer lock syringe type of syringe that allows a needle to be twisted onto the tip and then locked in place; provides a secure connection and prevents accidental removal of the needle 
vitreoretinal pertaining to the vitreous and the retina

Susvimo™ PI Explorer

PI Page 2 Flashcard Questions

Questions

1. What is the boxed warning associated with SUSVIMO?

2. What is the indication for SUSVIMO?

3. By whom should the procedures associated with insertion, refill, and removal of SUSVIMO be performed?

4. What is the recommended dose of SUSVIMO?

Susvimo™ PI Explorer

PI Page 2 Flashcard Answers

Questions

1. What is the boxed warning associated with SUSVIMO?

The SUSVIMO implant has been associated with a 3-fold higher rate of endophthalmitis than monthly intravitreal injections of ranibizumab. Many of these events were associated with conjunctival retractions or erosions. Appropriate conjunctiva management and early detection with surgical repair of conjunctival retractions or erosions may reduce the risk of endophthalmitis. In clinical trials, 2.0% of patients receiving a ranibizumab implant experienced at least one episode of endophthalmitis.

2. What is the indication for SUSVIMO?

SUSVIMO (ranibizumab injection) is indicated for the treatment of patients with nAMD who have previously responded to ≥ 2 intravitreal injections of a VEGF inhibitor medication.

3. By whom should the procedures associated with insertion, refill, and removal of SUSVIMO be performed?

SUSVIMO initial fill and implant insertion and removal procedures must be performed by an ophthalmologist experienced in vitreoretinal surgery and trained in SUSVIMO procedures. SUSVIMO refill-exchange procedures should be performed by an ophthalmologist trained in SUSVIMO refill-exchange procedures.

4. What is the recommended dose of SUSVIMO?

The recommended dose of SUSVIMO is 2 mg (0.02 mL of 100 mg/mL solution) continuously delivered via the permanent SUSVIMO ocular implant with refills administered every 24 weeks (approximately 6 months).

Susvimo™ PI Explorer

References

- American Society of Retinal Specialists. Intravitreal injections. <https://www.asrs.org/content/documents/fact-sheet-30-intravitreal-injections.pdf>
- Cioffi GA, Liebmann JM. Diseases of the visual system. Chapter 395. In: Goldman L, ed. *Goldman-Cecil Medicine*. 26th ed. Philadelphia, PA: Elsevier, Inc. 2020:2528–2535e2.
- Cordero I. Understanding and caring for an indirect ophthalmoscope. *Community Eye Health*. 2016;29(95):57.
- Eylea®. Prescribing Information. Regeneron Pharmaceuticals Inc.; 2021.
- Fogli S, Del Re M, Rofi E, Posarelli C, Figus M, Danesi R. Clinical pharmacology of intravitreal anti-VEGF drugs. *Eye (Lond)*. 2018;32(6):1010–1020. doi:10.1038/s41433-018-0021-7
- Food and Drug Administration. Guidance for Industry: Labeling for Human Prescription Drug and Biological Products – Implementing the PLR Content and Format Requirements. February 2013. Accessed June 9, 2021. <https://www.fda.gov/files/drugs/published/Labeling-for-Human-Prescription-Drug-and-Biological-Products---Implementing-the-PLR-Content-and-Format-Requirements.pdf>
- Friedman N, Kaiser P, Pineda II R. The Massachusetts Eye and Ear Infirmary Illustrated Manual of Ophthalmology. 4th ed. Philadelphia, PA: Elsevier, Inc. 2019.
- Kanski JJ, Bowling B. *Clinical Ophthalmology: A Systemic Approach*. 7th ed. Philadelphia, PA: Elsevier, Inc. 2011.
- LUCENTIS®. Prescribing Information. Genentech Inc.; 2018.
- Medisave. Needle and syringe information. Accessed June 15, 2021. <https://www.medisave.co.uk/needle-syringe-info>
- Medscape. B-scan ocular ultrasound. Accessed November 10, 2021. <https://emedicine.medscape.com/article/1228865-overview>
- Medscape. Ocular hypotony. Accessed November 10, 2021. <https://emedicine.medscape.com/article/1207657-overview>
- Passive transport. Accessed June 25, 2021. <https://biologydictionary.net/passive-transport/>
- Pawling Engineering Products. Septa. Accessed June 21, 2021. <https://www.pawlingep.com/products/septa>
- Ranibizumab (LUCENTIS®): Visual Impairment due to Choroidal Neovascularization Secondary to Pathologic Myopia [Internet]. Canadian Agency for Drugs and Technologies in Health. Published August 2015. Accessed June 17, 2021. <https://www.ncbi.nlm.nih.gov/books/NBK349531/>
- SUSVIMO™. Prescribing Information. Genentech Inc.; 2021.
- Taber's Online Medical Dictionary. Accessed June 27, 2021. www.tabers.com